

SEBORRHEIC DERMATITIS IN ADULTS

The clinical picture and course of seborrheic dermatitis in adults differ from that seen in infants.

Clinical Findings

Seborrheic eczematid is the mildest form of the disease (eczematid = eczema-like, dermatitis-like). It is associated with seborrhea, scaling, mild redness, and often pruritus affecting the scalp, eyebrows, nasolabial folds, retroauricular areas, sternum, and shoulder blades. Asymptomatic fluffy, white dandruff of the scalp—referred to as **pityriasis sicca**—represents the mild end of the seborrheic dermatitis spectrum. **Nasolabial erythema**, more common in young women than men, may also be part of this disease spectrum.

Patchy seborrheic dermatitis is the classic, chronic, and recurrent form of the disease. Lesions typically affect: - Scalp - Temples - Inner eyebrows and glabella - Nasolabial folds - Retroauricular folds and external ear canal - V-shaped areas of the upper chest and back

Less commonly involved sites include intertriginous areas such as the neck folds, axillae, submammary regions, umbilicus, and genitocrural folds.

Skin lesions are characterized by: - Yellowish color - Mild to severe erythema - Mild inflammatory infiltrate - Oily, thick scales and crusts (historically termed **pityriasis steatoides**)

Patients often report pruritus, especially on the scalp and in the ear canal. Lesions begin with follicular and perifollicular redness and papules, then expand into well-demarcated, round to circinate (petaloid) patches (from Greek *petalon*, meaning a thin plate or leaf).

The **pityriasisiform type** of seborrheic dermatitis occurs on the trunk and mimics **pityriasis rosea**, presenting with oval, scaly lesions whose long axes follow the rib contours.

In some individuals, only one or two body sites are affected. Chronic dermatitis of the ear canal may be the sole manifestation and is often misdiagnosed as a fungal infection. Another variant is **blepharitis**, characterized by honey-colored crusts along the eyelid margins and casts of keratinous debris around the eyelashes.

In men, a more **follicular variant** may involve large areas of the back, flanks, and abdomen.

Prognosis and Clinical Course

The disease typically persists for years to decades, with: - Improvement during warmer months - Exacerbations during colder seasons

Fluctuations in severity are common, and long-term management is often required.